

PATIENT INFORMATION

Name: Michael Carter

Age: 56

Sex: Male

Procedure: L3–L5 lumbar fusion with interbody spacers

Surgery date: October 8, 2026

Postoperative day: 1

Primary caregiver: Emily Carter, spouse

Caregiver phone: (202) 555-0143

Care setting: Inpatient surgical unit

SUBJECTIVE

Patient reports lower-back soreness around the surgical site, rated 4/10 at rest and 6/10 during transfers. Reports partial relief after prescribed pain medication. Denies new leg weakness, groin numbness, or loss of bladder or bowel control.

Patient tolerates food and fluids without vomiting. Reports passing gas but has not had a bowel movement since surgery. Voiding after urinary catheter removal.

Spouse is present and plans to assist with meals, transportation, household tasks, and medication organization after discharge.

OBJECTIVE

Vital signs: Temperature 98.6°F; blood pressure 128/76 mmHg; heart rate 78 beats/minute; respiratory rate 16 breaths/minute; oxygen saturation 97% on room air.

General: Alert, oriented, and participating in conversation.

Neurological examination: Lower-extremity strength 5/5 bilaterally in assessed muscle groups. Light-touch sensation intact bilaterally. No new neurological deficit identified in this fictional assessment.

Surgical site: Dressing clean, dry, and intact. No visible drainage through the dressing. Incision not directly inspected during this assessment.

Mobility: Transfers with standby assistance and walks approximately 60 feet using a walker. Requires reminders to avoid twisting during transfers.

ASSESSMENT

Postoperative day one following L3–L5 lumbar fusion with interbody spacers. Incisional pain limits movement but improves with prescribed treatment. No new neurological changes identified. Continued mobility training and discharge education are needed.

CAREGIVER AND HOME ASSESSMENT

Emily Carter is available to remain with the patient during the first 72 hours after discharge. She will drive him home and assist with daily activities.

Home has two entry steps and a bedroom and bathroom on the main floor. Stair training has not yet been completed. Spouse reports concern about assisting with transfers and requests additional instruction.

Patient and spouse prefer written instructions in English. Spouse will participate in discharge teaching with the patient's permission.

POSTOPERATIVE PLAN

October 9, 2026 — Postoperative Day 1

Continue pain management and neurological monitoring according to surgical-team orders. Continue physical and occupational therapy for transfers, walking, entry steps, and caregiver training.

Before discharge, confirm the surgeon's activity restrictions, lifting limit, brace requirements, wound-care instructions, and medication plan. Review the prescribed bowel regimen and monitor urinary and bowel function.

Provide Michael and Emily with follow-up details, the surgical team's contact information, and instructions for urgent symptoms.

Planned discharge is October 9 at 2:00 PM, contingent on final surgical-team assessment, safe mobility, and completion of caregiver education. Emily will provide transportation home.

October 10, 2026 — Postoperative Day 2

Sample postoperative telephone check-in at 10:00 AM to review pain control, medication understanding, wound concerns, mobility, urinary and bowel function, and caregiver questions. Continue prescribed care and discharge instructions.

October 11, 2026 — Postoperative Day 3

Continue prescribed postoperative care and caregiver assistance. Monitor for changes in symptoms, wound concerns, or difficulty following the discharge plan. Continue movement restrictions and the prescribed bowel regimen.

October 12, 2026 — Postoperative Day 4

Sample telephone check-in at 10:00 AM to review recovery and ongoing support needs. The first 72 hours after the planned discharge end at 2:00 PM. Continue restrictions and caregiver assistance as needed beyond this period.

October 22, 2026 — Two-Week Follow-Up

Sample appointment at 10:00 AM for wound assessment, pain and mobility review, and evaluation of sutures or staples if present.

November 19, 2026 — Six-Week Follow-Up

Sample appointment at 10:00 AM to review recovery, activity restrictions, and imaging if ordered by the surgeon.

All discharge times, check-ins, and appointments are fictional planning examples and remain subject to clinical confirmation.

Author: Jordan Lee, MD

Status: Signed